Wolters Kluwer

June 16, 2025

Response from Wolters Kluwer on the Request for Information
from CMS and ASTP/ONC on the Health Technology Ecosystem

Below is Wolters Kluwer's response to the Request for Information (RFI) recently issued by the Centers for Medicare and Medicaid Services (CMS) and the Assistant Secretary for Health Technology/Office of the National Coordinator for Health Information Technology (ASTP/ONC) on the *Health Technology Ecosystem* (file code CMS-0042-NC). Thanks in advance for considering our response.

As way of background, Wolters Kluwer is a leading global provider of clinical technology and evidence-based solutions that assist effective decision-making and outcomes across the healthcare continuum. Key solutions include UpToDate®, UpToDate® Lexidrug™, UpToDate® Patient Engagement, Medi-Span®, Sentri7®, Lippincott® Solutions, Ovid®, and Health Language®. Wolters Kluwer had annual revenue in 2024 of $6.4 billion.

To ease CMS and ASTP/ONC's review of our response, we provide a summary of our policy recommendations immediately below, accompanied by a citation of the RFI question/response where the recommendation is discussed. Our detailed responses further below address specific questions posed by CMS and ASTP/ONC, organized by RFI section.

Summary of Policy Recommendations

- Continue to build-out the **United States Core Data for Interoperability** (USCDI), with an emphasis on adding more granular data elements related to date and time (TD-7; TD-13)
- To facilitate provider access to cloud-based clinical decision support, the **CDS Hooks** standard should be added to the ONC Health IT Certification Program (PR-5)
- Given that provider trust is critical to adoption of **artificial Intelligence-enabled digital solutions** in APMs, we recommend keeping the transparency and disclosure requirements of the new *Decision Support Interventions* criterion in place (VB-2)
- CMS should consider adding more **digital health-related Improvement Activities to MIPS**, making reporting on the existing *Engage Patient and Families* Activity mandatory, or worthy of bonus scoring if chosen for reporting (PC-5)
- To aid in the development of a **patient-facing out-of-pocket calculator**, CMS and ASTP/ONC should explore ways to standardize payer benefit data elements (PC-5)
- Since **pharmacists** in many states are expanding the clinical services they provide to beneficiaries, particularly in rural areas, and are making good use of digital health tools, CMS should consider making pharmacists eligible for payment for select Part B services rendered to Medicare beneficiaries (PC-5)

Patients and Caregivers:

PC- 1. *What health management or care navigation apps would help you understand and manage your (or your loved ones') health needs, as well as the actions you should take?*

a. What are the top things you would like to be able to do for you or your loved ones' health that can be enabled by digital health products?

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

Wolters Kluwer's Response: One of the more important initiatives CMS and ASTP/ONC can undertake is to empower patients to be more active participants in their own care. Providing patients with access to their medical record or deploying apps that improve their care experience are important, but if a patient does not understand the nature of their illness or condition, and the affirmative steps they can take to manage or improve their condition, our health care system will continue to experience poor outcomes.

Patients and consumers increasingly turn to online sources and health apps when they need clarity, but these sources are not always reliable. To address this, technology developers and clinical content providers are beginning to develop and deploy software solutions and digital tools designed to empower patients with the latest medical evidence on their condition and provide actionable steps and recommendations on how they can manage or improve their conditions.

The optimal design for a patient empowerment app of the near-future is to integrate the patient's personal health record with relevant evidence-based health information and recommendations that augment (but not substitute for) their clinician's expertise and counsel. The design would also offer care navigation capability that aligns with the patient's health insurance coverage and benefits, and seamlessly interacts with the patient's care team for scheduling, questions or on-line consultations.

Such a tool would educate patients and their caregivers to easily understand their prescribed medications or durable medical equipment using simple descriptions, pictures or video. Accompanying health risk assessments should be presented in plain language, configurable based on a patient or caregiver's preference, offer suggested action steps the patient/caregiver can take, include a list of questions they can ask their clinician and provide benchmarks enabling them to compare themselves to similar patients. Benefit and price data would be analyzed at the point of care to give the patient and their provider a real-time estimate of the patient's out-of-pocket costs for possible treatment options. They would also be capable of real-time updating and reconciling after seeing a new provider, and enable the patient to share a portion or all their record with whichever provider or party they choose.

Digital tools designed for caregivers should possess many of the same features of the patient tool described above. Caregivers in a position to assist in care decisions should be provided specific questions to ask providers and information on what to expect in upcoming procedures, or with new medications. Similarly, information on when changes to a patient's condition might warrant escalation to a health care professional would also be helpful.

Clarity and context are just as important for caregivers as it is for patients. For example, in advance of a procedure, caregivers should be made privy to information about how the patient should prepare and offered a preview of best practices for a successful recovery. Educating patients and caregivers on proper medication use and side effects is critical to prevent misuse or accidents. Also, while it is appropriate and helpful to see the actual notes, values or results from a clinician encounter or lab test, the conclusions should be contextualized with respect to the outcome, impact, or risk, its significance (good/bad/neutral) and next steps.

PC-2. *c. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?*

Wolters Kluwer's Response: In general, data types are often available, but the ability to consolidate information from different providers into a consumer-facing app is lacking, forcing most patients and caregivers to log in to each provider's patient portal. Also, though many data types are available, as we mention above, they are not easily understood by patients and caregivers and therefore not very helpful.

PC-3. *Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?*

Wolters Kluwer's Response: There are numerous such apps available today, including ones from Wolters Kluwer. Our solutions provide access to a library of health information to support beneficiary health, including educational leaflets and videos, and medication leaflets, visuals, descriptors, consumer-facing drug monographs, audio-enabled leaflets, and video overviews of the medications.

For example, UpToDate Consumer Education builds off the trusted evidence-based content currently used by millions of clinicians worldwide to offer an extensive collection of health consumer content and APIs to support optimal engagement via web and digital experiences. Similarly, UpToDate Digital Medication Education offers a broad collection of consumer medication content, APIs, and continually updated, advanced digital medication leaflets with flexible delivery options to help enhance safety and convenience for patients.

Whether the topic is a patient's general health, a specific condition, or the medication they take, it is important for the content to reflect the latest scientific evidence, delivered in easily understood language that addresses the reasons behind a care recommendation.

PC-4. *What features are missing from apps you use or that you are aware of today?*

Wolters Kluwer's Response: We outlined a possible future-state for health apps in our response to PC-1 that enable patients and caregivers to successfully navigate a fragmented care system and be more active partners with their care team. In general, we believe vendors in health care should aim to educate, empower, and enable a patient to be an equal participant in their own health, in ways that both promote evidence-based best practices and understanding, as well as enabling a person's individual

choice and ability to weigh their options in partnership (as they define it) with their chosen providers/care team.

PC–5. *What can CMS and its partners do to encourage patient and caregiver interest in digital health products?*

Wolters Kluwer's Response: At the May 20th webinar and June 3rd meetings to discuss the RFI, CMS announced its intention to pursue several initiatives that could help spur patient and caregiver use of digital health tools. These include building an interoperable national provider directory, developing an identity verification process for medicare.gov, expanding access to Blue Button 2.0 and possibly committing the agency to involvement in TEFCA. We look forward to reviewing additional details on these initiatives as each moves forward.

There are several actions CMS can also take that will have downstream impacts in the expanded use of digital health products. For example, CMS' various initiatives on hospital and (possibly) drug price transparency will help provide valuable data points for shared decision-making between patients, their caregivers and providers. Medicare Advantage plans can also play a constructive role leveraging their new Patient APIs, and CMS might consider requiring Plans to send out annual reminders to their enrollees about API access to their claims history and the utility this provides for helping manage their health.

Since patients often want to understand their out-of-pocket costs (OOPC) when contemplating an upcoming procedure, CMS and ASTP/ONC should explore ways to standardize payer benefit information so it can be aligned with pricing data and presented to patients as an OOPC calculator. This might be accomplished by building out the *Health Insurance* Data Class of USCDI, or adding a proposed calculator as a permissible use of technology to an upcoming Medicare Advantage-based pilot launched by the Center for Medicare and Medicaid Innovation.

Getting Part B providers to encourage patients and caregivers to use digital tools could also be greatly facilitated by leveraging the Merit-based Incentive Payment System (MIPS) Improvement Activity reporting category. We note that currently only one Improvement Activity, *Engage Patients and Families to Guide Improvement in the System of Care,* specifically calls out the use of digital health tools, and reporting on the Activity is voluntary. CMS might consider adding more digital health-related Improvement Activities to MIPS, making reporting on the existing *Engage Patient and Families* Activity mandatory, or worthy of bonus scoring if chosen for reporting.

Another approach might target pharmacists, who are playing a more active role on patient care teams, particularly in rural areas that are plagued by provider shortages. Many pharmacists are embracing their expanded roles and leveraging digital health tools to enhance patient literacy and medication adherence. With many retail pharmacies now on the front line of patient care, it is important that pharmacists have access to the patient's health record to be able to deliver a better, more holistic experience and help assist the patient in their care. CMS could help facilitate bigger roles for pharmacists by including them in new initiatives aimed at improving interoperability and exchanging health data. Pharmacists would be deserving of being included in the planned provider directory. CMS should also

consider recognizing pharmacists as 'providers' eligible for payment for clinical services under Part B that they are licensed to provide in their state.

PC-6. *What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?*

Wolters Kluwer's Response: An important feature to foster accessibility and use of digital health tools is to make medical content available in plain language and at an appropriate reading level. Tools should also be available across the range of digital formats, including phone apps, websites and SMS. As an example of how this might be accomplished, we refer CMS and ASTP/ONC to a draft rule issued by FDA in 2023 that would have simplified the one-page leaflet that accompanies prescription drug orders. While still not finalized, the proposed leaflet format contained many insights about making content about prescriptions simple and accessible to patients (with digital as an acceptable format), insights that are equally applicable to other forms of health content.

PC-8. *In your experience, what health data is readily available and valuable to patients or their caregivers or both?*
a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

Wolters Kluwer's Response: Claims data is readily available but lacks important clinical context. For example, laboratory data is recorded in CPT with no results attached. To be useful to patients and caregivers, that data needs to be merged with laboratory results data. LOINC is the interoperability standard for laboratory results but is often not leveraged when sharing data between healthcare entities. While not the ideal state, post-processing of the data collected at the point of record can allow for the normalization of uncodified data, but that data needs to be complete in order to be accurate. To normalize data to LOINC, it must contain as many of the six axes of the standard as are applicable (e.g., *Component, Property, Time Aspect, System, Scale, Method*). We note this same principle applies to quality reporting.

We appreciate the efforts of President Trump to advance health care price transparency during his first Administration and now in his second. As CMS has learned in its implementation of the hospital price transparency regulations, pricing data is difficult to obtain and interpret prior to services being rendered. This data also needs to be understood by patients and their caregivers in order to be valuable. Standardized procedure names that are easily understood by the general population remain elusive in the industry and reaching consensus on such names would allow for significant progress on price transparency.

Patient-generated data, such as patient history forms or remote monitoring data, is valuable but not accessible in EHR data. For example, patient-reported data **(**e.g., from MyChart) is underutilized and not consistently pulled into data sets. CMS should consider incentivizing the inclusion of patient-generated health data as a valuable source. Barriers include both data collection and technical limitations in pulling from patient-facing systems.

ONC Health IT Certification program requirements and the USCDI have standardized data collection at the point of care but are not yet robust enough to capture details found in rich encounter notes. For example, in the context of public health, data is only valuable and shareable when standardized across public health agencies. We have not yet reached that point, with the result that merged data from multiple sources is difficult to analyze for individual patient care or advancements in public health. Public health requirements also vary by state in the content and standards that are required for important use cases such as vaccines administration or syndromic surveillance.

Providers

PR-5. *Which of the following FHIR APIs and capabilities do you already support or utilize in your provider's organization's systems, directly or through an intermediary. For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:*
i. CDS Hooks (for clinical decision support integrations)

Wolters Kluwer's Response: We are pleased that CMS and ASTP/ONC included CDS Hooks among the FHIR APIs under consideration. When the Health Information for Economic and Clinical Health (HITECH) Act was passed in 2011, lawmakers expected it would result in widespread adoption of electronic health record technology by medical providers, which would, in turn, lead to substantial cost savings, improvements in care and the avoidance of unnecessary tests and preventable medical errors. With more than 95% of hospitals and 80% of clinician practices now using EHR technology certified to ONC standards, HITECH has largely achieved the goal of widespread adoption by providers, but cost savings and improved health outcomes attributable to the use of health technology have proved elusive. We believe widespread adoption and use of CDS Hooks is one way to help deliver on these still-unmet goals of HITECH.

CDS Hooks enables improved and broader use of health data with clinical decision support engines to support a growing list of use cases within the clinical workflow. Cloud-based and scalable, use of CDS Hooks can help accelerate improved adherence to clinical guidelines and medical evidence, and reduce medication errors and variability in care. As such, we strongly supported the addition of the CDS Hooks standard to the ONC Health IT Certification Program when it was proposed in the HTI-2 draft rule.

The addition of CDS Hooks to the Certification program makes the most sense if it is tied to one or more use cases. Fortunately, CDS Hooks supports several, including the following:

- Real-time Prescription Benefit Tracking;
- Preventive care reminders;
- Evidence-based treatment suggestions (e.g. we noticed this patient has high BP and we would suggest medication XYZ);
- Evidence-based treatment alternatives (e.g. instead of medication XYZ that you ordered, you might consider medication ABC because it's more effective and/or safer for this patient);
- Appropriate use of diagnostic tests;
- Prior authorization;
- Pharmacogenomics;
- Next generation medication safety screening (e.g., drug-drug interaction checking that evaluates patient-specific information to provide more tailored alerts)

In proposing the addition of CDS Hooks in the HTI-2 draft rule, ASTP/ONC envisioned a core implementation of the new standard, requiring support for registration, authorization, prefetch, card

display, internal apps (or "absolute url," which is the more technically meaningful term), SMART apps and launch context. As part of this initial implementation, ASTP/ONC was prepared to require support for two hooks, *patient-view* and *order-sign*, both of which are FHIR maturity model level 5.

With one exception, we supported all aspects of this initial implementation. *Patient-view* and *order-sign* are both foundational hooks necessary for accessing cloud-based decision support services across the full range of use cases supported by CDS Hooks. Our one concern was that ASTP/ONC did not also include the *order-select* hook in the initial implementation.

The challenge with only supporting the *order-sign* hook for the ordering workflow is that the clinician will only be accessing the clinical decision support services much later in their workflow, when a treatment decision may have already been made. If the CDS service then offers up a different treatment option, one the clinician might prefer, they will need to go back and re-enter the alternative order details regarding dose, route, frequency, refills and pharmacy location (note different details would be required for lab tests or imaging). Similarly, if the clinician had already made a mental commitment to their first choice, or informed the patient of the treatment choice, they would be less likely to change their treatment decision suggested by this late alert.

As ASTP/ONC is acutely aware, an ongoing complaint clinicians have with EHR technology is the interruptions often created in their workflows. The cognitive burden is even higher with *order-sign* alerts because the interruptions occur later in the workflow than with *order-select* alerts. Adding *order-select* as a required hook for initial implementation will help address this concern. *Order-select* enables the clinician to access CDS services earlier in the workflow, often before a treatment decision has been made. With the CDS output before them, the clinician can then make the treatment decision and move to *order-sign*, with no need to backtrack in their workflow, and at a workflow step where they may be more open to considering alternative recommendations.

Epic has the largest market share of any EHR vendor and already supports CDS Hooks *patient-view*, *order-sign* <u>and</u> *order-select*. While many other EHR vendors have yet to support CDS Hooks, they are generally committed to offering solutions that minimize user burden and align with clinical workflow. Once required to support CDS Hooks as part of the Certification Program, many will likely configure their support to include *order-select* to optimize the user experience, with or without a mandate to do so from ASTP/ONC.

For future rulemaking related to CDS Hooks, ASTP/ONC should also consider adding support for a very important capability of the standard – namely *suggestions* for alternative orders. This leads to a better user experience, reduction in user burden and, with the presentation of evidence-based treatment alternatives, better health outcomes for the patient. We also recommend eventual support for the CDS Hooks "*feedback*" capability, which provides valuable information from clinicians to CDS service providers on usage, effectiveness and efficiency of their clinical recommendations.

PR-9. *b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?*

Wolters Kluwer's Response: Patient privacy and data sharing regulations vary by state and in some cases can be more restrictive than HIPAA requirements. Protection is also provided by the federal Health Breach Notification Rule. We refer to our response in PC-1 where we discussed the ideal set of capabilities for digital health apps that support patients and caregivers. In general, data segmentation

remains a challenge in healthcare technology. CMS and ASTP/ONC should focus on strategies that incentivize the development and implementation of more robust data segmentation capabilities that allow for patients to consent at different levels of sharing for different types of data.

Technology Vendors, Data Providers and Networks

TD-2. *Regarding CMS Data, to stimulate developer interest—*
a. What additional data would be most valuable if made available through CMS APIs?
b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?
c. What obstacles prevent accessing these data sources today?
d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

Wolters Kluwer's Response: To foster semantic interoperability between payers and providers, CMS and ASTP/ONC should consider viewing patient data through a more expansive lens. This would include data that might traditionally be considered non-health related (e.g., access to healthy food, availability of transportation to a doctor's appointment). We understand the Administration is contemplating a multi-year project to eventually make all relevant federal data on beneficiaries available through a single government portal. This is an idea worthy of further exploration.

Regarding obstacles to data access, we refer to our response to PC-8 and would also add data segmentation challenges, granular consent management, the lack of semantic normalization in data being shared and the lack of standardization in bulk exports.

TD-7. *To what degree has USCDI improved interoperability and exchange, and what are its limitations?*
a. Does it contain the full extent of data elements you need?

Wolters Kluwer's Response: We strongly recommend the Trump Administration continue to build out the USCDI data classes and elements, which enables improved interoperability and exchange of health information data. USCDI was created during President Trump's first term as a way for the Administration and private industry to work together to identify the critical data needed to enable interoperability and achieve the goal that all patient health information be capable of exchange. Under the President's leadership, Version 1 of USCDI was finalized and officially added to the ONC Health IT Certification Program in 2020.

As President Trump begins his second term in office, his Administration is embarking on a campaign to fundamentally reframe how America's health care system operates, and to reorient that system to focus on the prevention of chronic diseases and empower patients to be more active participants in their health. Enhanced data exchange facilitated by broader adoption and use of USCDI will be vital to support the *Make America Healthy Again* transformation, helping providers and patients with shared decision-making, educating patients on how to optimize their health and ultimately leading to improved health outcomes. USCDI will also be instrumental in advancing two other priorities of the Administration: enabling new research into the causes of chronic disease; and assisting in the development and deployment of artificial intelligence in care delivery. On the latter, we note that Large Language Models perform best when built upon codified data, especially when building cohorts that the model relies on.

As we have commented in the past to ASTP/ONC on the various versions of USCDI, we strongly recommend adding data elements that address date and timing. We previously supported the addition of the *Performance Time* data element in the *Procedures* data class but continue to maintain it is not, as ASTP/ONC seems to believe, a multi-purpose data element that could apply across data classes.

Several other data classes in USCDI still need specific data elements that capture date and timing. Nowhere is this more important than with the *Medication* class, in which it is critical to provide attending clinicians with granular data on their patient's medications, not only to facilitate smooth continuity of care but to safeguard patient safety. As such, the timing of when medications were prescribed and subsequently administered are vital data points that should be included. This would include *Date Medication Prescribed* and *Date Medication Administered*.

Similar arguments can be made regarding the status and timing of vaccines, and we urge ASTP/ONC to also include *Immunization Status* and *Vaccination Administration Date*. Separately, ASTP also proposed to include in the *USCDI+ Quality* data set several date-related elements for the *Allergies and Intolerances* data class, including *Last Occurrence, Onset Time,* and *Recorded Date*. We supported the inclusion of all three in the *Quality* data set and would also support their addition to the next version of USCDI. Additional timing-related data elements that should be added include *Vital Sign Results: Date and Timestamps, Laboratory Results: Date and Time Stamps* and *Laboratory Test Performed Date.* The latter two data elements are Level 2 because of their widespread adoption and therefore represent minimal implementation burden to providers and developers.

TD-9 *- What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality? What would be the drawbacks?*

Wolters Kluwer's Response: Redefining the ONC Certification Program to prioritize API-enabled capabilities offers several advantages. It would allow data sharing to scale while also enabling better monitoring of information blocking, thereby reducing its incidence. Expanding the types of data shared would give providers a more complete view of their patients' health history and condition, leading to more holistic care plans and better outcomes. An API-enabled focus would also foster innovation, improve data quality and completeness, and empower patients to exercise greater control of their data through segmentation and granular consent.

TD-13. *b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?*

Wolters Kluwer's Response: As we note above, the USCDI has been a significant driver of capturing standardized data at the point of care. This remains critically important for priority use cases needed to inform patient care and care coordination. CMS and ASTP/ONC should continue to focus on this important driver of semantic interoperability. In efforts to date to build out the USCDI, the Health Information Technology Advisory Committee (HITAC) has made valuable contributions, and we urge ASTP/ONC to reconvene HITAC. While EHI export is valuable, barriers exist to standardizing the format and content of the export itself until true semantic interoperability can be fully realized through advancing technologies.

This is not an either-or question. The USCDI focuses on the core data needed for the highest priority use cases and helps provide important structured and codified data for those use cases. The data in the EHI Export provides additional and valuable information that is not codified. Even with advanced technologies, structured and codified data is imperative for accurate analysis and data-driven insights. We therefore need to continue to advance the standardization of the EHI export with the goal of a fully standardized and computable format AND continue to advance the USCDI to capture the information that is important to patient care use cases.

F. Value-based Organizations

VB-2. *How can key themes and technologies such as* ***artificial intelligence****, population health analytics, risk stratification, care coordination, usability, quality measurement and patient engagement be better integrated into APM requirements?*

Wolters Kluwer's Response: We recommend CMS and ASTP/ONC avoid the creation of overt APM requirements for integrating artificial intelligence (AI)-enabled technology. New entrants may be discouraged from participating in APMs if AI deployment is viewed as a prerequisite. Also, given that APM participants are at significant (if not full) financial risk for the underlying costs of their patient population, we believe they will make their own internal decisions on how best to deploy AI-enabled solutions that can improve clinical workflows, optimize health outcomes and drive cost efficiency.

The best role CMS, ASTP/ONC and other HHS agencies can play in facilitating the adoption of AI-enabled solutions is to help build trust in such solutions. A survey conducted in late 2024 by Wolters Kluwer found that 40% of the U.S. physicians polled were now ready to use GenAI when interacting with patients at the point-of-care. The findings reflect a rapid acceptance of the new technology more broadly, with 68% saying they have recently changed their views and are now more likely to think that GenAI would be beneficial to healthcare.

However, the survey also revealed that physicians will not blindly follow GenAI recommendations and that they would be selective when evaluating and implementing GenAI tools. For example, before relying on GenAI tools for assistance in clinical decision-making, 91% of respondents indicated they would need to know that GenAI-sourced materials were created by doctors and medical experts. Similarly, 89% reported that they would require vendors to be transparent about where information came from, who created it, and how it was sourced.

If these survey results are indicative of the wider provider community, and we believe they are, clinicians will not use GenAI-enabled tools at the point of care unless they understand how such solutions were developed and trust their output.

The recently implemented *Decision Support Interventions* (DSI) criterion was essentially designed to promote trust among medical professionals and spur adoption of AI tools used at the point of care. The new criterion includes transparency (or "source attribute") disclosures for DSI used in or with certified technology, as well as additional disclosures related to risk mitigation for "predictive" DSI.

We appreciate that CMS and ASTP/ONC are actively searching for existing regulations to rescind, thereby lightening the administrative and compliance burden on private entities, a goal we generally support in this context. Regarding the new DSI criterion, however, the Administration may want to take a wait-and-see approach. Compliance with the DSI criterion is technically voluntary for developers, not a legal requirement. Moreover, we consider its burden manageable, merely requiring disclosures of information related to the design, development, testing, validation and ongoing monitoring of the software's performance and, in the case of Predictive DSIs, risk mitigation practices.

Moreover, per our recent physician survey that underscores the importance of trust, even in the absence of the DSI criterion, such disclosures would almost certainly be requested (and likely required to be made) by hospitals, health systems and clinics participating in APMs before they commit to purchasing and deploying DSI solutions.

Should CMS and ASTP/ONC decide to leave the DSI criterion in place for now, HHS might consider issuing a Request for Information at the 12- or 18-month mark of implementation (i.e., January or July 2026). The RFI could seek public input on whether the 31 required source attributes are excessive and assess if the disclosed information effectively fosters trust and adoption among clinician end users.

We appreciate the opportunity to comment. If CMS or ASTP/ONC have questions or would like to discuss our comments in more detail, please contact Bob Hussey at bob@bobhussey.com.